

HOW TO OPEN A Home Care Agency in New Jersey

A step-by-step startup guide to the non-medical Health Care Service Firm — registration, certified aides, and how you get paid

Regulatory basis: N.J.A.C. 13:45B (Health Care Service Firms; Subchapters 13 & 14) · N.J.S.A. 34:8-43 et seq. (Health Care Service Firm Act) · New Jersey Board of Nursing (CHHA certification) · Medicaid MLTSS under NJ FamilyCare
Oversight: New Jersey Division of Consumer Affairs, Regulated Business Section (Department of Law & Public Safety)

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The Big Picture

New Jersey regulates home care in two very different ways, and knowing which lane you are in is the first and most important decision you will make. Skilled, medical home health — nursing, therapy, physician-ordered clinical care billed to Medicare — is a licensed home health agency under N.J.A.C. 8:42A, licensed by the Department of Health and requiring a certificate of need. Non-medical home care — personal care, homemaker, companion, and home health aide services that help a person live safely at home — is regulated completely differently: you register as a Health Care Service Firm with the New Jersey Division of Consumer Affairs. No Department of Health license, and no certificate of need.

This guide is about the second lane — the non-medical Health Care Service Firm, or HCSF — because it is the accessible entry point for a new owner. You register rather than get licensed, there is no certificate-of-need bottleneck, and you can build from a modest start. That does not mean it is unregulated: New Jersey has real, specific rules that protect vulnerable clients, and two of them make the state genuinely distinct.

First, your home health aides must be certified. A New Jersey home health aide is a certified homemaker-home health aide (CHHA), certified by the New Jersey Board of Nursing, and must be directly employed by your firm — not treated as an independent contractor. Second, New Jersey runs on registered-nurse delegation: a registered professional nurse serving as your health care practitioner supervisor assesses clients, delegates client-care tasks to your aides, verifies each aide is competent, and documents every delegated task in the client's record. A CHHA performs nursing tasks delegated through the authority of that nurse. Build your agency around those two facts and the rest of this guide falls into place. It is general educational information, not legal advice; confirm every step, figure, and contact with the Division of Consumer Affairs and the Board of Nursing before you rely on it.

Two Lanes — Which One Are You In?

Question	Non-Medical (this guide)	Skilled / Medical
What you provide	Personal care, homemaker, companion, home health aide services; delegated tasks under RN supervision	Skilled nursing, therapy, physician-ordered clinical care
How you are authorized	Register as a Health Care Service Firm with the Division of Consumer Affairs	Licensed home health agency (N.J.A.C. 8:42A) by the Department of Health
Certificate of need?	No	Yes — required
Typical payers	Private pay, Medicaid MLTSS personal care, long-term-care insurance	Medicare, Medicaid, insurance

The Path at a Glance

1. Write a business plan and set up the business — form your entity with the Division of Revenue & Enterprise Services (DORES), get your EIN, and line up insurance and workers' compensation.
2. Line up your registered professional nurse — the health care practitioner supervisor who will assess clients, delegate tasks, and supervise aides. Nothing runs without this role.
3. Secure your \$10,000 surety bond (required if your net worth is under \$100,000), or prepare the certified financial report that supports a waiver.
4. Prepare your written policies and procedures — client care, complaint handling, and personnel — which the Division expects to see.
5. Set up criminal background screening through IdentoGO for every direct-care employee, and a process to verify each aide's CHHA certification with the Board of Nursing.
6. Register as a Health Care Service Firm with the Division of Consumer Affairs, Regulated Business Section — application plus the \$500 fee — and pass the location inspection.
7. Hire and onboard your certified aides as employees, verify certifications, complete screening, and document competency for delegated tasks.
8. If you will serve Medicaid clients, obtain accreditation within one year and contract with the NJ FamilyCare managed care organizations for MLTSS; set up Electronic Visit Verification.
9. Open, market to referral sources, and run the compliance rhythm — renew your registration annually, keep certifications and the bond current, and prepare for your triennial audit.

Part 1 — Set Up the Business

Before you register with the state, stand up the business itself. Choose a legal structure — most home care owners form a limited liability company (LLC) or a corporation — and record the entity with the New Jersey Division of Revenue & Enterprise Services (DORES). Get your federal employer identification number (EIN) from the IRS, register for state employer obligations (including unemployment and temporary disability), and set up payroll, because in New Jersey your aides are employees, not contractors.

Put your insurance in place: general and professional liability appropriate to home care, and workers' compensation, which New Jersey requires once you have employees. Set up a real recordkeeping system — client files, personnel files, scheduling, and documentation — because the Division can inspect your business location and will expect client and employee documents to be secured. A well-considered business plan is not legally required, but it is strongly recommended and there are free resources (the Small Business Administration and SCORE) to help you write one.

Starting Costs — What to Budget For

Costs vary widely with your size, location, and how you staff. The figures below are the fixed, verifiable state-side items; the rest depends on your market. Confirm every current amount before you rely on it.

Item	What to expect
HCSF application fee	\$500 with your registration application (verify current amount)
Surety bond	\$10,000 bond required if net worth is under \$100,000; you pay a percentage premium, not the full amount. Waivable with a certified financial report showing net worth of \$100,000 or more.
Entity formation (DORES)	State filing fee to form your LLC or corporation (verify current amount)
Criminal background / fingerprinting	Per-employee IdentoGO fingerprinting fee for each direct-care hire (verify current amount)
RN supervisor	Salary or contracted cost of your registered professional nurse — your largest fixed people cost
Insurance	Liability and workers' compensation premiums (quote for your situation)
Accreditation (if Medicaid)	Cost of accreditation from a recognized body, required within one year if you serve Medicaid clients

Part 2 — The People: Certified Aides and Your RN Supervisor

New Jersey's home care model is built on two roles, and getting them right is what separates a compliant agency from a risky one.

Certified Homemaker-Home Health Aides (CHHAs)

Your home health aides must hold current certification as homemaker-home health aides through the New Jersey Board of Nursing, and they must be directly employed by your firm. You verify each aide's certification before you assign them and monitor it so it never lapses — you can confirm certification status with the Board's License Verification Section. You may also employ companions and homemakers who are not certified aides, but if you send a worker who is not a CHHA to provide home-based services, you must give the client written notice, on your letterhead, that the individual is not a certified aide. That non-CHHA disclosure is a specific New Jersey rule; build it into your intake paperwork so you never miss it.

The Registered Professional Nurse Supervisor

A registered professional nurse licensed in New Jersey serves as your health care practitioner supervisor. This nurse assesses each client and helps build the service plan, delegates client-care tasks to your aides only after confirming the aide is competent to perform them for that specific client, is available to your aides for direction, and — critically — documents in the client's record every task delegated to an aide. This is not a formality you can skip: the delegation-and-documentation model is the legal backbone of how your aides are allowed to provide care. Plan for enough nurse-supervisor capacity to match your client volume from day one.

Every direct-care employee is screened before serving clients through the State's fingerprint-based criminal background check via IdentoGO, and your officers, directors, principals, and owners provide the criminal-conviction certifications the registration requires. Orient and train every aide before independent duty, and keep personnel records — certification verification, screening, orientation, competency, and supervisory documentation — for each one.

Part 3 — Register With the Division of Consumer Affairs

With your nurse, bond, policies, and screening process ready, you register as a Health Care Service Firm with the New Jersey Division of Consumer Affairs, Regulated Business Section, through its online portal. The application is submitted with a \$500 fee and includes your firm and ownership information, the criminal-conviction certifications, your surety bond (or waiver documentation), and proof of licensure for your registered nurse supervisor. After the application is received, the Division inspects your business location to confirm it complies with the rules — including that client and employee documents are properly secured — so have your office and recordkeeping set up before you apply.

Registration is not one-and-done. It expires annually and you renew it through the same portal; operating on a lapsed registration carries a penalty of \$500 per day, so track your renewal date carefully. New Jersey also requires a Health Care Service Firm to submit an audit by a certified public accountant in the third calendar year after registration and every third year after that — plan for that triennial audit as a standing obligation, not a surprise.

Registration Snapshot

Item	Detail	Where
Who registers you	Division of Consumer Affairs, Regulated Business Section	(973) 504-6370
Application fee	\$500 (verify current)	DCA online portal
Renewal	Annual; \$500/day penalty if lapsed	Same portal
Audit	CPA audit in year 3 and every 3 years after	Submit to the Director

Part 4 — How You Actually Get Paid

Home care revenue in New Jersey comes from three main places, and most agencies use more than one. Be honest with yourself about your payer mix, because it drives your cash flow and your compliance obligations.

Private Pay

Many non-medical home care clients pay privately, by the hour, out of income, savings, or family support. You set your own rates for your market; disclose them clearly in a written service agreement, including any minimums and cancellation terms. Private pay is the simplest to start with because it does not require accreditation or a managed-care contract — but confirm current market rates in your area rather than assuming a number.

Medicaid MLTSS (Managed Long Term Services and Supports)

New Jersey's Medicaid program pays for personal care and related home-care services for eligible clients through Managed Long Term Services and Supports, delivered by the NJ FamilyCare managed care organizations. To participate, you must obtain accreditation from a recognized accrediting body within one year of registration, then contract with the managed care organizations. Rates are set through those contracts and by the program, so confirm current rates directly with the managed care organizations rather than relying on any figure quoted elsewhere. Once enrolled, you verify each client's authorization, deliver only authorized services, and complete Electronic Visit Verification for applicable visits.

Long-Term-Care Insurance

Some clients have long-term-care insurance that reimburses non-medical home care. Confirm the policy's covered services, any authorization it requires, and how it pays, and document services to that standard.

A word on honesty about numbers: this guide deliberately does not quote a private-pay hourly rate or an MLTSS reimbursement rate, because both vary by market and by managed-care contract and change over time. Get your real numbers from the managed care organizations and from your own market research before you build a budget on them.

Common Mistakes to Avoid

- \ Treating aides as independent contractors. New Jersey requires that your certified aides be directly employed by the firm. Misclassifying them is a compliance and tax problem.
- \ Skipping the non-CHHA disclosure. If you send a companion or homemaker who is not a certified aide, you must give the client written notice on letterhead. Build it into intake.
- \ Under-resourcing the RN supervisor. The delegation-and-documentation model is the legal basis for your aides' work. Too little nurse capacity is a direct compliance risk.
- \ Assuming you need a Department of Health license or a certificate of need. Non-medical home care registers with the Division of Consumer Affairs — do not confuse it with a licensed home health agency.
- \ Believing the bond is \$50,000. Some sites say so; the rule sets a \$10,000 bond (waivable at \$100,000 net worth). Confirm the current requirement.
- \ Letting registration or certifications lapse. Registration renews annually with a \$500/day penalty for operating lapsed, and an aide whose CHHA certification expires cannot be assigned.
- \ Forgetting the triennial audit and the one-year accreditation deadline for Medicaid. Both are standing obligations — calendar them.
- \ Providing skilled, physician-ordered clinical care. That crosses into licensed-home-health territory. Stay within your non-medical scope and delegated tasks, and refer out when a client needs more.

Your First 90 Days

A realistic sequence once you have decided to move forward. Adjust to your situation, and confirm current requirements as you go.

10. Days 1–15: Form your entity with DORES, get your EIN, open business banking, and start your business plan. Begin recruiting your registered professional nurse supervisor.
11. Days 15–45: Secure your surety bond, quote and bind insurance and workers' compensation, set up your office and secure recordkeeping, and draft your written policies and procedures.
12. Days 30–60: Set up IdentoGO screening and your CHHA verification process, begin recruiting aides, and prepare your registration application and supporting documents.
13. Days 45–75: Submit your Health Care Service Firm registration with the \$500 fee, prepare for the location inspection, and finalize onboarding, orientation, and competency processes.
14. Days 60–90: Once registered, hire and onboard aides, launch marketing to referral sources, and — if serving Medicaid — begin your accreditation process and managed-care contracting, and set up Electronic Visit Verification.

Who to Call — Two Metro Startup Call Lists

These are real, verified starting points for two of New Jersey's largest home care markets. County offices handle Medicaid intake and aging-services referrals that feed home care clients to you. Phone numbers and offices change — verify each before you rely on it.

Essex County (Newark and surrounding)

Office	Purpose	Phone
Essex County Division of Family Assistance & Benefits (Board of Social Services)	Medicaid / NJ FamilyCare intake	(973) 395-8300
Essex County Division of Senior Services (ADRC)	Aging & Disability Resource Connection; client referrals	(973) 395-8375
Newark Department of Health & Community Wellness	Local health / community resources	(973) 733-7604

Hudson County (Jersey City and surrounding)

Office	Purpose	Phone
Hudson County Department of Family Services (Board of Social Services)	Medicaid / NJ FamilyCare intake	(201) 420-3000
Hudson County Office on Aging (ADRC)	Aging & Disability Resource Connection; client referrals	(201) 369-4313
NJ FamilyCare (statewide enrollment)	Confirm client Medicaid eligibility	1-800-701-0710

Statewide Contacts

Agency	For	Contact
DCA Regulated Business Section	HCSF registration & status	(973) 504-6370
New Jersey Board of Nursing	CHHA certification	(973) 504-6430
Board of Nursing — License Verification	Verify an aide's certification	(973) 273-8090
Division of Revenue & Enterprise Services	Form your LLC / corporation	(609) 292-9292
IdentoGO (MorphoTrust)	Fingerprint background checks	1-877-503-5981
NJ FamilyCare	Medicaid eligibility & MLTSS	1-800-701-0710
Adult Protective Services (statewide)	Report abuse/neglect of a vulnerable adult	1-800-792-8820
SBA / SCORE New Jersey	Free business-plan help	sba.gov / score.org

Mailing addresses: Division of Consumer Affairs, Regulated Business Section, 124 Halsey Street, 7th Floor, P.O. Box 45028, Newark, NJ 07101. New Jersey Board of Nursing, P.O. Box 45010, Newark, NJ 07101.

Glossary

Health Care Service Firm (HCSF): A business that employs or refers workers to provide non-medical home care — personal care, homemaker, companion, and home health aide services — registered with the Division of Consumer Affairs.

CHHA: Certified homemaker-home health aide; a home health aide certified by the New Jersey Board of Nursing, employed by the firm, performing tasks delegated by a registered professional nurse.

Health care practitioner supervisor: The registered professional nurse who assesses clients, delegates tasks, verifies aide competency, and documents every delegated task in the client's record.

Non-CHHA disclosure: The written notice, on letterhead, a firm must give a client when it sends a worker who is not a certified aide (N.J.A.C. 13:45B-13.8).

Surety bond: The \$10,000 bond a firm files (if net worth is under \$100,000) to secure compliance with the Act; waivable with a certified financial report.

MLTSS: Managed Long Term Services and Supports; New Jersey's Medicaid managed-care program that pays for personal care and related home-care services through managed care organizations.

Electronic Visit Verification (EVV): The electronic system that verifies the date, time, location, and services of Medicaid-reimbursed home visits.

Accreditation: Accreditation from a recognized body, required within one year of registration to participate in Medicaid home care.

Certificate of need (CON): A Department of Health approval required for a licensed home health agency — not required for a non-medical Health Care Service Firm.

A Final Word on Compliance

New Jersey's Health Care Service Firm rules exist to protect vulnerable people receiving care in their own homes, and the Division of Consumer Affairs enforces them through registration, inspection, and oversight, with the Board of Nursing standing behind aide certification. Treat compliance as the foundation of your business, not a hurdle: employ and properly screen your aides, resource your registered nurse supervisor, honor the non-CHHA disclosure, keep your registration and bond current, meet your accreditation and audit obligations, and report and correct problems promptly.

This guide is general educational information based on New Jersey requirements at the time of writing, not legal advice. Rules, fees, figures, and contacts change. Confirm every step with the New Jersey Division of Consumer Affairs, the New Jersey Board of Nursing, and NJ FamilyCare, and consider professional guidance, before you rely on anything here.